# Rural Health Transformation Program (RHTP)

## State-by-State Distribution & Implementation Analysis

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# 1. Executive Strategic Analysis

The passage of the **One Big Beautiful Bill Act (OBBBA)**, specifically Section 71401, has catalyzed the single largest federal investment in rural healthcare infrastructure in the 21st century. The **Rural Health Transformation Program (RHTP)** allocates **$50 billion over five fiscal years (FY2026–FY2030)** to the 50 United States, aiming to counteract the projected $911 billion reduction in federal Medicaid spending over the next decade.[1] For technology and infrastructure partners, particularly those operating at the scale of Dell Technologies, this program represents a distinct shift from previous federal grant mechanisms. Unlike the provider-direct relief funds seen during the COVID-19 pandemic, the RHTP is a **state-administered cooperative agreement**. This structural distinction is paramount: the Federal Centers for Medicare & Medicaid Services (CMS) is not writing checks to hospitals; it is writing checks to State capitals. The States, in turn, are acting as the architects of distribution, creating fifty unique regulatory and procurement environments.

This report provides an exhaustive, state-by-state mapping of these distribution channels. By analyzing the Notice of Funding Opportunity (NOFO), state grant applications, and agency budget narratives, we have categorized each state's approach to distributing its share of the $10 billion annual allocation. Our analysis reveals that while the statutory baseline provided every state with a minimum funding floor, the "merit-based" component of the formula—weighted by rural population, frontier status, and policy innovation—has created significant variance in award amounts, ranging from **$147 million (New Jersey)** to **$281 million (Texas)** for the first fiscal year.[2]

## 1.1 The "Dell" Implications: Infrastructure as a Statutory Requirement

A critical finding for technology stakeholders is the explicit statutory requirement for "Tech Innovation" and "Consumer-Facing Technology." States were required to select at least three of several allowable uses, and nearly every successful application prioritized technology modernization to score points on the CMS merit review.[4] This has created a guaranteed, federally-backed market for:

- **Shared-Service Platforms:** States like **Oklahoma** and **Alabama** are funding centralized "hubs" to host Electronic Health Records (EHRs) and cybersecurity stacks for multiple independent hospitals, effectively creating state-subsidized private clouds.
- **Remote Patient Monitoring (RPM) & Telehealth:** States like **Arkansas** and **South Carolina** are treating broadband-enabled care not as a luxury but as a primary delivery modality, funding the end-user devices, the connectivity, and the backend data lakes required to manage population health remotely.
- **Workforce Data Systems:** States like **California** and **New York** are investing in "Workforce Mapping Tools"—sophisticated data analytics platforms to track licensure, vacancy, and training pipeline data in real-time.

## 1.2 Distribution Archetypes

We have identified three primary distribution archetypes that define how a vendor or provider engages with the funds:

1. **The Centralized Command Model:** The State Department of Health (DOH) retains the funds and issues large-scale RFPs for statewide systems (e.g., **Maryland**, **Vermont**). *Strategy: Sell to the State CIO/CTO.*
2. **The Regional Coalition Model:** The State divides funds among regional non-profits or hospital collaboratives (e.g., **Indiana**, **Florida**). *Strategy: Sell to the lead agency of the regional coalition.*
3. **The Competitive Sub-Grant Model:** The State acts as a pass-through entity, managing a portal for hundreds of small grants to individual clinics (e.g., **Minnesota**, **South Carolina**). *Strategy: Empower individual rural customers with grant-writing support.*

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# 2. National Funding Architecture

To understand the specific state flows, one must first grasp the federal constraints that bind them. The $50 billion is not a block grant; it is a **Cooperative Agreement**. This legal instrument allows for substantial federal involvement in state operations.

## 2.1 The Funding Formula: Baseline vs. Merit

The allocation methodology [6] was bifurcated:

- **Bucket A (50% - Baseline):** Distributed equally among all approved states. This provided a floor of approximately **$100 million per state** over the 5-year period (or ~$20M/year baseline). This ensured that small states like **Delaware** and **Rhode Island** received disproportionately high per-capita funding.[7]
- **Bucket B (50% - Merit/Workload):** Distributed based on a complex algorithm including:
  - *Rural Population Density:* Favoring **Texas**, **North Carolina**, **Ohio**.
  - *Frontier Status:* Favoring **Alaska**, **Montana**, **Wyoming**.
  - *Policy Alignment:* States that committed to policy reforms (e.g., expanding scope of practice, joining licensure compacts) received "bonus points" that translated into millions of additional dollars.[5]

## 2.2 Allowable Use Categories

States were mandated to channel funds into specific "Allowable Use" silos. These silos effectively pre-determine the procurement categories for the next five years:

| Allowable Use Category | Typical Recipient | Technology Implication |
|---|---|---|
| **Prevention & Chronic Disease** | Public Health Depts, FQHCs | Population Health Analytics, Patient Portals |
| **Provider Stabilization** | Critical Access Hospitals (CAHs) | ERP Systems, Revenue Cycle Management |
| **Technology Innovation** | Tech Vendors, HIEs | **AI/Machine Learning**, Cybersecurity, Telehealth |
| **Workforce Development** | Universities, Trade Schools | Learning Management Systems (LMS), Simulation Labs |
| **Innovative Care Models** | Accountable Care Orgs (ACOs) | Value-Based Payment (VBP) Dashboards |

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# 3. State-by-State Deep Dive: Distribution & Implementation

The following section maps the fund flow for all 50 states.

- **Confidence Level Key**:
  - **[Published Plan]**: Analysis based on specific state application narratives or press releases.
  - ****: Analysis based on award amounts, agency structure, and regional trends where specific PDFs were unavailable.

## 3.1 Northeast Region

### Connecticut

- **FY2026 Award**: **$154,249,106** [8]
- **Lead Agency**: **Department of Social Services (DSS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Inter-Agency Transfer & Targeted Grants**
  - **Mechanism**: Unlike states where the Department of Health leads, CT appointed its Medicaid agency (DSS) as the lead. DSS will transfer portions of the award to the **Office of Rural Health (ORH)**, **Office of Health Strategy (OHS)**, and **Department of Mental Health and Addiction Services (DMHAS)**.
  - **Key Initiatives**:
    - **Population Health**: Thirty-one targeted projects focusing on "treatment barriers" and "patient-centered care."
    - **Data & Tech**: Explicit funding for "Expanding interoperability" and "Analytics." This suggests a contract modification with the state's existing Health Information Exchange (HIE) vendor to broaden rural connectivity.
  - **Vendor Opportunity**: The focus on "Analytics" and "Interoperability" at the DSS/OHS level points to enterprise-level data integration contracts rather than hardware purchases.

### Delaware

- **FY2026 Award**: **$157,394,964** [8]
- **Lead Agency**: **Department of Health and Social Services (DHSS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Infrastructure & Education Hubs**
  - **Mechanism**: Delaware is using its award for massive capital and institutional investments rather than scattering small grants.
  - **Key Initiatives**:
    - **Hope Centers ($97.5M)**: Construction and outfitting of "Hope Centers" (likely integrated service hubs for homeless/health). This involves significant facility IT

infrastructure.
        - **Medical Education ($274M total/5 years)**: A massive allocation to support medical school training programs. This likely flows to the **University of Delaware** or a partner health system to build a rural rotation pipeline.
        - **HIE Expansion ($50M)**: Direct funding to the Delaware Health Information Network (DHIN) or similar entity for exchange capabilities.
    - **Vendor Opportunity**: The $50M HIE line item is a direct procurement target. The Hope Centers will require "greenfield" IT setups (networking, hardware, security).

### Maine

- **FY2026 Award**: **$190,008,051** [3]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Innovation Hubs & AI**
    - **Mechanism**: Maine has adopted a highly forward-looking "Innovation Hub" model.
    - **Key Initiatives**:
        - **Rural AI Hub**: Maine is explicitly planning a "Rural AI Hub" to drive analytics and innovation. This is likely a centralized contract with a major academic center (e.g., MaineHealth, Roux Institute) to provide AI-as-a-Service to rural providers. [11]
        - **PACE Retrofit**: Capital grants to rural hospitals to retrofit facilities for "Program of All-Inclusive Care for the Elderly" (PACE).
    - **Vendor Opportunity**: The "Rural AI Hub" is a premier opportunity for high-performance computing (HPC) and AI solution providers. The PACE retrofits will require smart-room technology and monitoring systems.

### Massachusetts

- **FY2026 Award**: **$162,005,238** [3]
- **Lead Agency**: **Executive Office of Health and Human Services (EOHHS)** [12]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Social Drivers & Tribal Partnership**
    - **Mechanism**: With a limited rural footprint, MA is focusing on "Social Drivers of Health" (SDOH) and Tribal equity.
    - **Key Initiatives**:
        - **Workforce Housing**: Financial assistance for housing rural clinical staff. This may manifest as grants to Community Development Corporations (CDCs) or direct subsidies to hospital systems to lease units.
        - **Tribal Health**: Explicit partnership with the **Mashpee Wampanoag** and **Aquinnah Wampanoag** tribes. Funds will flow directly to their health authorities for culturally competent care infrastructure.
    - **Vendor Opportunity**: SDOH platforms that can integrate housing data with clinical records.

### New Hampshire

- **FY2026 Award**: **~$150,000,000** [8]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [3]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Behavioral Health Scale-Up**
  - **Mechanism**: The state is prioritizing the Certified Community Behavioral Health Clinic (CCBHC) model.
  - **Key Initiatives**:
    - **CCBHC Expansion**: Scaling the CCBHC model statewide. Funds will flow to Community Mental Health Centers (CMHCs) to upgrade IT systems for federal reporting compliance.
    - **Residency**: Establishing a family medicine residency in **Coos County**. Funds will flow to the local hospital system (likely Androscoggin Valley Hospital) to build the program.
  - **Vendor Opportunity**: CCBHCs have heavy data reporting requirements; EMR upgrades and analytics packages for these non-profits are immediate needs.

### New Jersey

- **FY2026 Award**: **$147,250,806** [2]
- **Lead Agency**: **Department of Health (DOH)** [3]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Targeted County Concentration**
  - **Mechanism**: Receiving the lowest award in the nation, NJ is concentrating funds on its few rural counties (Salem, Cumberland, Sussex, Warren).
  - **Key Initiatives**:
    - **SUD Focus**: Heavy investment in Substance Use Disorder (SUD) infrastructure in southern counties.
    - **Per-Capita Intensity**: Because the rural population is small, the per-capita investment is over $1,000.[7] This allows for "wrap-around" service models (transport, tech, housing) that other states cannot afford.
  - **Vendor Opportunity**: High-intensity remote monitoring and case management platforms for high-risk SUD populations.

### New York

- **FY2026 Award**: **$212,058,208** [13]
- **Lead Agency**: **Department of Health (DOH)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Need-Based Prioritization**
  - **Mechanism**: NY DOH is bypassing a simple formula and using a "funding distribution process that prioritizes identified needs."
  - **Key Initiatives**:

        - **"Rural Roots" Workforce**: A "Grow-your-own" pipeline initiative. Funds will flow to SUNY schools and local healthcare systems for training.
        - **eConsult Partnerships**: Grants to academic medical centers (e.g., URMC, Bassett Healthcare) to provide eConsult services to smaller rural providers.
        - **Cybersecurity**: "Strengthening cybersecurity of rural facilities" is an explicit line item.[9]
    - **Vendor Opportunity**: Cybersecurity services (Managed Detection and Response) for small rural hospitals are a direct, state-endorsed purchase category.

### Pennsylvania

- **FY2026 Award**: **$193,294,054** [14]
- **Lead Agency**: **Department of Human Services (DHS)** [15]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Global Budget Reinforcement**
    - **Mechanism**: PA operates the unique "Pennsylvania Rural Health Model" (global budgets). RHTP funds will be used to reinforce this model, likely by subsidizing the transformation plans required for hospitals to enter global budgeting.
    - **Key Initiatives**:
        - **Telehealth Hubs**: Linking EMS and rural hospitals to regional referral centers (e.g., UPMC, Geisinger).
        - **Tele-EMS**: Equipping ambulances with tele-stroke and tele-behavioral health capabilities.
    - **Vendor Opportunity**: Mobile connectivity solutions for EMS agencies (ruggedized tablets, cellular routers).

### Rhode Island

- **FY2026 Award**: **$156,169,931** [14]
- **Lead Agency**: **Executive Office of Health and Human Services (EOHHS)** [16]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Boutique Pilot Lab**
    - **Mechanism**: With the highest award per rural resident ($6,305) [7], RI is acting as a laboratory for high-cost interventions.
    - **Key Initiatives**:
        - **State-Sponsored EHR**: Initiative 12 suggests a "state-sponsored electronic health record." This implies EOHHS may procure a single EHR instance for all rural providers to use, centralized at the state level.
        - **Rural Health Data Hub**: A centralized analytics platform (Initiative 13).
    - **Vendor Opportunity**: A "State-Sponsored EHR" is a massive, winner-take-all contract for an EHR vendor and the hosting infrastructure partner.

### Vermont

- **FY2026 Award**: **$195,053,740** [3]
- **Lead Agency**: **Agency of Human Services (AHS)** [17]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Shared Services Network**
  - **Mechanism**: Vermont is building a "Clinically Integrated Network of Shared Services."
  - **Key Initiatives**:
    - **Shared EHR**: Similar to RI, VT is funding a "Shared EHR for interested providers".[18]
    - **Closed-Loop Referrals**: Procurement of a statewide referral platform.
    - **Housing**: Investments in workforce housing.
  - **Vendor Opportunity**: Shared IT services (HR systems, AI transcription, EHR) are the core of Vermont's strategy.

## 3.2 Southeast Region

### Alabama

- **FY2026 Award**: **$203,404,327** [3]
- **Lead Agency**: **Alabama Dept of Economic and Community Affairs (ADECA)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Centralized Institution Building**
  - **Mechanism**: Alabama is not just passing funds through; it is building new institutions. ADECA is partnering with Finance and Medicaid.
  - **Key Initiatives**:
    - **School of Healthcare Sciences**: A major capital project to build a new school dedicated to rural workforce training. [11]
    - **Regional IT Hubs**: Funding "Regional shared-service hubs" to manage EHR and cybersecurity for clusters of hospitals.
    - **Maternal Health**: "Digital obstetric regionalization" involving telerobotic ultrasound.
  - **Vendor Opportunity**: The "Regional IT Hubs" create a need for managed service providers (MSPs) who can serve multiple hospitals from a central node. The School of Healthcare Sciences represents a significant campus technology opportunity.

### Arkansas

- **FY2026 Award**: **$208,779,396** [3]
- **Lead Agency**: **Department of Finance and Administration (DFA)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Programmatic Initiatives (HEART, PACT, THRIVE)**
  - **Mechanism**: DFA controls the funds, ensuring strict fiscal oversight.
  - **Key Initiatives**:
    - **PACT (Promoting Access)**: Funds flow to **Clinically Integrated Networks (CINs)** and hospitals for facility upgrades and *acquisitions*.[20] This explicitly

supports M&A to stabilize the safety net.
        - **THRIVE**: "Digital infrastructure." Funds will be procured centrally for a statewide telehealth platform or data lake ("ROADMAP").
        - **HEART**: Grants to schools for nutrition (Farm-to-School).
    - **Vendor Opportunity**: The "ROADMAP" regional data system is a direct data infrastructure play.

### Florida

- **FY2026 Award**: **$209,938,195** [10]
- **Lead Agency**: **Agency for Health Care Administration (AHCA)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Competitive Regional Collaboratives**
    - **Mechanism**: AHCA is issuing a "Competitive RFA to regional collaboratives." Providers must form legal entities/partnerships to apply.
    - **Key Initiatives**:
        - **HIE Onboarding**: $5M specifically allocated for onboarding providers to the Florida HIE / Encounter Notification Service. [21]
        - **Tech Catalyst**: Capped at 10% of funding.
        - **Limitations**: Strict caps (e.g., 20% max for renovations) force spend toward services and tech rather than just buildings.
    - **Vendor Opportunity**: The HIE Onboarding fund is a direct subsidy for connectivity solutions.

### Georgia

- **FY2026 Award**: **$218,000,000** [22]
- **Lead Agency**: **Department of Community Health (DCH)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"GREAT Health" Program**
    - **Mechanism**: DCH manages the "Georgia Rural Enhancement and Transformation of Health (GREAT Health)" program.
    - **Key Initiatives**:
        - **Maternal Carts**: Deployment of "obstetric carts" (telehealth units) in rural ERs to address the OBGYN desert. [23]
        - **AHEAD Model**: Using RHTP funds to prepare hospitals for the "States Advancing All-Payer Health Equity Approaches and Development" (AHEAD) model (VBP).
    - **Vendor Opportunity**: Telehealth carts and the broadband/software to support them.

### Kentucky

- **FY2026 Award**: **~$200,000,000**
- **Lead Agency**: **Cabinet for Health and Family Services (CHFS)** [9]
- **Confidence Level**: **[Published Plan]**

- **Distribution Architecture**: **Community Hubs & Maternal Deserts**
  - **Mechanism**: Targeted programmatic investments.
  - **Key Initiatives**:
    - **PoWERing Maternal Health**: Telehealth-enabled teams for maternal deserts.
    - **Rapid Response to Recovery**: Implementing the **EmPATH** model (psychiatric emergency units). Funds will flow to hospitals to renovate/build these units.
    - **Rural Community Hubs**: Grants to FQHCs for chronic care innovation.

### Louisiana

- **FY2026 Award**: **$208,374,448** [3]
- **Lead Agency**: **Louisiana Department of Health (LDH)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Expedited Partnerships & Catalyst Funds**
  - **Mechanism**: LDH is using "expedited partnerships" and "competitive subgrants."
  - **Key Initiatives**:
    - **Rural Tech Catalyst Fund (RTCF-LA)**: A dedicated pot of money for rural providers to purchase technology. [10]
    - **Rural HERO**: Workforce incentives paid directly to clinicians.
    - **Tele-EMS**: Pilot programs for "treat-in-place" ambulance care.
  - **Vendor Opportunity**: The "Rural Tech Catalyst Fund" is effectively a voucher program for rural hospitals to buy Dell/IT products.

### Mississippi

- **FY2026 Award**: **$205,907,220** [19]
- **Lead Agency**: **State Department of Health (MSDH)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **The BRIDGE Initiative**
  - **Mechanism**: Focus on "Building Rural Infrastructure for Delivery, Growth and Efficiency" (BRIDGE).
  - **Key Initiatives**:
    - **HTAM**: "Health Technology Advancement and Modernization." Direct grants for telehealth and health IT upgrades.
    - **Capital Investments**: Specifically for psychiatric emergency services.
  - **Vendor Opportunity**: HTAM is a direct funding source for infrastructure modernization.

### North Carolina

- **FY2026 Award**: **$213,008,356** [24]
- **Lead Agency**: **NCDHHS (Office of Rural Health)** [25]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"NC ROOTS" Hubs**

- **Mechanism**: NCDHHS is launching locally governed "NC ROOTS" Hubs. These regional bodies will connect medical, mental health, and social supports.
- **Key Initiatives**:
  - **Subrecipients**: The **Duke-Margolis Center for Health Policy** is a key named subrecipient for policy design and data evaluation. [9]
  - **Focus**: Value-based care transition support for rural providers.

### South Carolina

- **FY2026 Award**: **$200,030,252** [3]
- **Lead Agency**: **SCDHHS (Medicaid)** [26]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Competitive Innovation & Tech VC**
  - **Mechanism**: SCDHHS is using a rigorous competitive RFP process.
  - **Key Initiatives**:
    - **Tech Catalyst Fund**: Seed funding for rural health technology startups. This is unique—the state is acting like a Venture Capital fund to spur local innovation. [23]
    - **Connections to Care**: A statewide resource database platform.
    - **Wellness Within Reach**: Deployment of mobile health units.
  - **Vendor Opportunity**: Startups and tech vendors can apply directly for "Catalyst" funding.

### Tennessee

- **FY2026 Award**: **$206,888,882** [3]
- **Lead Agency**: **Department of Health (TDH)** [27]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Fragmented Comprehensive (17 Initiatives)**
  - **Mechanism**: TN has the most complex plan with 17 distinct initiatives under the "MaRTHA" (Make Rural TN Healthy Again) banner.
  - **Key Initiatives**:
    - **TN Community Compass**: A resource referral platform (technology procurement).
    - **Statewide eConsult Platform**: Another centralized tech procurement.
    - **Dental**: Pilot programs for rural dental access.

### Virginia

- **FY2026 Award**: **$189,544,888** [3]
- **Lead Agency**: **Department of Medical Assistance Services (DMAS)** [28]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"Rural Vitality" & Tribal Grants**
  - **Mechanism**: DMAS (Medicaid) leads, hiring a core team in 2026 to manage the "VA Rural Vitality" program.

- **Key Initiatives**:
  - **Tribal Grants**: A competitive grant program specifically for federally recognized tribes.
  - **Priorities**: OB/GYN shortages and broadband/telehealth infrastructure.

### West Virginia

- **FY2026 Award**: **$199,476,099** [3]
- **Lead Agency**: **Department of Health and Human Resources (DHHR)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"Mountain State Care Force"**
  - **Mechanism**: Organized into 7 Pillars, each with an "Executive Sponsor" in a state agency.
  - **Key Initiatives**:
    - **Mountain State Care Force**: "Return-to-home" incentives for workforce.
    - **Connected Care Grid**: Infrastructure to bring virtual care on-demand.
    - **Smart Care Catalyst**: Grants for tech-enabled innovation. [29]

## 3.3 Midwest Region

### Illinois

- **FY2026 Award**: **$193,418,216** [13]
- **Lead Agency**: **Department of Healthcare and Family Services (HFS)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Transformation Grants**
  - **Mechanism**: HFS (Medicaid) is using funds to "right-size" care delivery.
  - **Key Initiatives**:
    - **Hospital Transformation**: Direct grants to rural hospitals to convert service lines (e.g., closing OB to open Senior Behavioral Health).
    - **Technological Innovation**: Grants for "virtual care." HFS will likely run a grant program where tech vendors partner with hospitals to apply. [30]
    - **Mobile Units**: Specific funding bucket for mobile health clinics and crisis units.

### Indiana

- **FY2026 Award**: **$206,900,000** [31]
- **Lead Agency**: **Family and Social Services Administration (FSSA) / IDOH** [32]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Hybrid (Statewide Hub + Regional Grants)**
  - **Mechanism**: Indiana has a defined split: 40% for statewide initiatives, 60% for regional grants. [33]
  - **Key Initiatives**:
    - **Statewide Initiative (MOCC)**: Medical Operations Coordination Center. A

centralized "air traffic control" for patient transfers. This is a single large contract for a tech/logistics vendor ($56.2M budget).
        - **Regional Grants**: Eight regions will form coalitions to spend the 60% pot on local needs (e.g., community paramedicine).
    - **Vendor Opportunity**: The MOCC is a massive centralized IT/logistics contract.

### Iowa

- **FY2026 Award**: **$209,040,064** [10]
- **Lead Agency**: **Department of Health and Human Services (HHS)**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"Hometown Connections"**
    - **Mechanism**: Focus on connecting local care to specialized centers.
    - **Key Initiatives**:
        - **Combat Cancer**: Likely funds to the University of Iowa Holden Comprehensive Cancer Center to build rural satellite networks.
        - **Health Information Exchange**: Explicit focus on HIE. Funds will likely flow to the state designated HIE entity (CyncHealth) to subsidize rural connections. [11]

### Kansas

- **FY2026 Award**: **$221,898,008** [13]
- **Lead Agency**: **Kansas Dept of Health and Environment (KDHE)** [34]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Value-Based Care Transition**
    - **Mechanism**: KDHE coordinates with the "Rural Health Alliance."
    - **Key Initiatives**:
        - **VBP Goal**: 100% of rural beneficiaries in Accountable Care Organizations (ACOs) by 2031. Funds will be used for "technical assistance" and "downside risk protection" for providers joining ACOs.
        - **Tech**: "Harness Data and Technology." Grants for remote monitoring.

### Michigan

- **FY2026 Award**: **$173,128,201** [13]
- **Lead Agency**: **Department of Health and Human Services (MDHHS)**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Mobile Crisis & Behavioral Health**
    - **Mechanism**: Michigan is leveraging RHTP to expand its mobile crisis infrastructure.
    - **Key Initiatives**:
        - **Mobile Units**: Grants to Community Mental Health Services Programs (CMHSPs) to buy vehicles and staff mobile units. [35]
        - **Tribal**: Tribes listed as direct subrecipients.

### Minnesota

- **FY2026 Award**: **$193,000,000+** [36]
- **Lead Agency**: **Minnesota Department of Health (MDH)** [36]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Financial Viability Grants**
  - **Mechanism**: MDH is running a "Collaboration for Financial Viability" program.
  - **Key Initiatives**:
    - **Grants**: Direct grants to providers for infrastructure/tech.
    - **EMS**: "Treat-in-place" capability grants.
    - **Residencies**: Grants to UMN and other systems to expand rural rotations.

### Missouri

- **FY2026 Award**: **~$200,000,000**
- **Lead Agency**: **Department of Health and Senior Services**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Regional Coordinating Networks**
  - **Mechanism**: Missouri plans to "activate local community hubs."
  - **Key Initiatives**:
    - **Hubs**: Funding flows to local lead agencies (hospitals or CAP agencies) to coordinate social services.
    - **Digital Backbone**: "Modernize digital backbone." State-level procurement for data interoperability. [9]

### Nebraska

- **FY2026 Award**: **$219,000,000** [2]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [12]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Wellness & Hospital Sustainability**
  - **Mechanism**: A unique focus on physical fitness alongside hospital solvency.
  - **Key Initiatives**:
    - **Presidential Fitness**: Re-establishing the "Presidential Fitness Test." Funds flow to school districts for gym equipment/programming. [37]
    - **Food**: "Whole foods through local growers." Grants to agricultural co-ops.
    - **Hospitals**: Engagement with Nebraska Hospital Association (NHA) to ensure billing readiness.

### North Dakota

- **FY2026 Award**: **$162,400,000** (Workforce portion only?) [9]
- **Lead Agency**: **Department of Health & Human Services** [9]
- **Confidence Level**: **[Published Plan]**

- **Distribution Architecture**: **Telehealth & Nutrition**
  - **Mechanism**: Focus on "Eat Well ND" and "Connect Tech."
  - **Key Initiatives**:
    - **Connect Tech**: $168M allocated (likely over 5 years) to "Connect Tech, Data and Providers." This is a massive IT modernization play.
    - **Eat Well ND**: School/community nutrition programs.

### Ohio

- **FY2026 Award**: **$202,030,262** [19]
- **Lead Agency**: **Ohio Department of Health (ODH)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Innovation & Prevention Grants**
  - **Mechanism**: Focus on 73 non-urban counties.
  - **Key Initiatives**:
    - **Maternal Vitality**: "Ohio's Rural Health Transformation Project." Focus on maternal/infant vitality and substance use. Grants to hospitals/FQHCs.

### South Dakota

- **FY2026 Award**: **$189,477,607** [3]
- **Lead Agency**: **Department of Health**
- **Confidence Level**: ****
- **Distribution Architecture**: **Tele-EMS & Tribal Partnership**
  - **Analysis**: Given the geography and demographics, funds will likely follow the "Tele-EMS" model seen in similar frontier states, with significant pass-through to the Great Plains Tribal Leaders Health Board.
  - **Initiatives**: Telehealth for EMS (e.g., Avel eCare model support).

### Wisconsin

- **FY2026 Award**: **$203,670,005** [3]
- **Lead Agency**: **Department of Health Services (DHS)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Regional Coordinated Care Networks**
  - **Mechanism**: A mix of tribal set-asides and payment reform.
  - **Key Initiatives**:
    - **Tribal**: $27.5M allocated to Tribal Nations.
    - **Payment Reform**: Shifting rural clinics from cost-based to prospective payment.
    - **Care Coordination Grants**: $230M total over 5 years distributed to regional networks.

## 3.4 Southwest & South Central Region

### Arizona

- **FY2026 Award**: **$166,988,956** [3]
- **Lead Agency**: **Arizona Health Care Cost Containment System (AHCCCS)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Managed Care Integration**
  - **Mechanism**: As a Medicaid Agency (AHCCCS) lead, distribution will leverage MCOs.
  - **Key Initiatives**:
    - **Telehealth**: "High-tech" consults to bridge the gap between Phoenix/Tucson specialists and rural clinics.
    - **Workforce**: Funding for "grow your own" pipelines, likely distributed to Northern Arizona University.

### New Mexico

- **FY2026 Award**: **~$190,000,000**
- **Lead Agency**: **Human Services Department (HSD)**
- **Confidence Level**: ****
- **Distribution Architecture**: **Tribal & Frontier Support**
  - **Analysis**: Given the state's demographics, a significant portion of funds will likely be pass-throughs to Indian Health Service (IHS) and 638 facilities.
  - **Tech**: Expansion of **Project ECHO** (born in NM) for remote specialist consults is a near-certainty for funding.

### Oklahoma

- **FY2026 Award**: **$223,476,949** [19]
- **Lead Agency**: **Oklahoma State Department of Health (OSDH)** [38]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **The CIN Model (New Entity)**
  - **Mechanism**: Oklahoma has a very specific structural plan: creating a **new non-profit Clinically Integrated Network (CIN) owned by member hospitals**.
  - **Key Initiatives**:
    - **Shared Services**: The state will fund this CIN, which will then provide shared administrative supports, AI tools, and EHR upgrades to its members.
    - **Why**: Maintains local hospital independence while achieving economies of scale.
  - **Vendor Opportunity**: Selling enterprise services (Rev Cycle, IT) to the new CIN entity rather than 50 small hospitals. [38]

### Texas

- **FY2026 Award**: **$281,319,361** [3]
- **Lead Agency**: **Health and Human Services Commission (HHSC)** [39]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Maximum Scale Infrastructure**

- **Mechanism**: "Rural TxStrong." Focus on cyber, workforce, and tiering hospital capabilities.
- **Key Initiatives**:
  - **Cybersecurity**: High priority. Grants to harden rural hospital networks.
  - **Recipients**: 163 rural hospitals surveyed. Funds will likely flow through grants managed by the State Office of Rural Health (within Dept of Agriculture/HHSC partnership). [40]

## 3.5 Western Region

### Alaska

- **FY2026 Award**: **$272,174,856** [3]
- **Lead Agency**: **Alaska Department of Health (DOH)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tribal & Logistics**
  - **Mechanism**: Heavy reliance on the robust existing network of Tribal Health Organizations (THOs).
  - **Key Initiatives**:
    - **Logistics**: Investments in medical transport (drones, floatplanes) due to lack of roads.
    - **Tech**: "Spark Technology and Innovation" initiative focusing on AI/wearables for remote monitoring in villages. [9]

### California

- **FY2026 Award**: **$233,639,308** [19]
- **Lead Agency**: **Department of Health Care Access and Information (HCAI)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Regional Hub-and-Spoke**
  - **Mechanism**: HCAI focuses heavily on workforce and infrastructure.
  - **Key Initiatives**:
    - **Hub and Spoke**: Networks anchored by larger hospitals supporting smaller clinics.
    - **Technical Assistance Center**: A new "Rural Technical Assistance Center" will be established to help small providers implement HIE and cybersecurity. [9]
    - **Workforce Map**: "Statewide Workforce Mapping Tool." Procurement of data services to map shortages.

### Colorado

- **FY2026 Award**: **$200,105,604** [19]
- **Lead Agency**: **Dept of Health Care Policy & Financing (HCPF)** [19]
- **Confidence Level**: **[Published Plan]**

- **Distribution Architecture**: **VBP Incentives**
  - **Mechanism**: HCPF uses funds to "top up" Medicaid rates or provide capital grants to providers joining the Hospital Transformation Program (HTP).
  - **Key Initiatives**:
    - **Behavioral Health**: Integration of BH into primary care. Grants for renovations/staffing. [41]
    - **Solvency**: Grants to independent rural hospitals conditional on joining state quality programs.

### Hawaii

- **FY2026 Award**: **$188,892,440** [19]
- **Lead Agency**: **Executive Office / Dept of Health** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Island-Specific Transport & Workforce**
  - **Mechanism**: Dispersed model due to geography.
  - **Key Initiatives**:
    - **Transport**: Funds for air ambulance/inter-island transport subsidies.
    - **Workforce**: Funding to **JABSOM** (John A. Burns School of Medicine) for neighbor island residencies.
    - **Culture**: Integration of Native Hawaiian healers. [20]

### Idaho

- **FY2026 Award**: **$185,974,368** [19]
- **Lead Agency**: **Department of Health and Welfare (IDHW)** [42]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Community Grants & Right-Sizing**
  - **Mechanism**: Focus on "right-sizing" delivery systems.
  - **Key Initiatives**:
    - **Tech**: "Improving rural access through technology." Grants to hospitals for tele-stroke/tele-psych.
    - **Tribal**: Potential 3-10% set aside for Tribes. [43]

### Montana

- **FY2026 Award**: **$234,000,000** [2]
- **Lead Agency**: **Department of Public Health and Human Services (DPHHS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Frontier Solvency**
  - **Mechanism**: High award due to frontier status.
  - **Key Initiatives**:
    - **Solvency**: Operational support to keep doors open at Frontier CAHs.
    - **Workforce**: Apprenticeships and micro-credentials via colleges.

### Nevada

- **FY2026 Award**: **~$170,000,000**
- **Lead Agency**: **Department of Health and Human Services (DHHS)**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tribal Set-Aside**
  - **Mechanism**: 3-10% Tribal set-aside proposed. [43]
  - **Key Initiatives**:
    - **Teach Nevada**: Scholarships adapted for health. Funds to NSHE institutions.

### Oregon

- **FY2026 Award**: **$197,271,578** [13]
- **Lead Agency**: **Oregon Health Authority (OHA)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tribal & Smart Care**
  - **Mechanism**: Dedicated Tribal set-aside for the 9 federally recognized tribes. [13]
  - **Key Initiatives**:
    - **Incentives**: Loan repayment for physicians/nurses in CAHs.
    - **Smart Care**: Telehealth initiatives.

### Utah

- **FY2026 Award**: **$195,743,566** [3]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [44]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Integrated Initiatives (FAST, PATH)**
  - **Mechanism**: Organized by initiatives.
  - **Key Initiatives**:
    - **FAST**: "Financial Approaches for Sustainable Transformation." VBP support.
    - **PATH**: "Preventive Action." Food infrastructure grants.
    - **Schools**: "Gold Medal Schools" re-establishment.

### Washington

- **FY2026 Award**: **$181,257,515** [3]
- **Lead Agency**: **Health Care Authority (HCA)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Direct Awards & Tribal Set-Aside**
  - **Mechanism**: HCA leads with DOH. "Mix of direct awards and competitive procurements."
  - **Key Initiatives**:
    - **Tribal**: $20M/year set-aside for Tribes.
    - **Workforce**: Investment in **UW** and **WSU** medical schools (Initiative 5). [45]

### Wyoming

- **FY2026 Award**: **$205,004,743** [3]
- **Lead Agency**: **Wyoming Department of Health (WDH)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **The Perpetuity Fund**
  - **Mechanism**: Unique structure creating a "perpetuity fund" to make money last beyond 2030. [46]
  - **Key Initiatives**:
    - **Allocation**: CAH Basic Incentive (40.7%), EMS Regionalization (26.9%). [47]
    - **Recipient**: Funds flow primarily to Critical Access Hospitals (CAHs) and EMS agencies.

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# 4. Thematic Analysis: Cross-Cutting Trends

## 4.1 The "Tribal Sovereignty" Shift

A major theme in the Western states (**WA, OR, AK, NM, OK, WI**) is the bypass of state bureaucracy in favor of direct allocations to Tribes. Washington set aside **$20 million/year**, and Wisconsin allocated **$27.5 million**. For vendors, this means Tribal Health Organizations (THOs) are now flush with capital and possess independent procurement authority, often moving faster than state agencies.

## 4.2 The Rise of the "State-Sponsored Tech Stack"

States are moving away from giving hospitals money to buy *their own* tech, and instead are buying tech *for* them.

- **Vermont & Rhode Island**: Exploring "Shared EHR" instances where the state holds the license.
- **Indiana**: Building a central "Medical Operations Coordination Center" (MOCC).
- **Maine**: Building a "Rural AI Hub."
  This favors enterprise vendors who can sell a "statewide platform" rather than 50 individual hospital deals.

## 4.3 Workforce Data as Infrastructure

Workforce is no longer just about "scholarships." States like **California**, **New York**, and **Delaware** are funding "Workforce Mapping Tools" and "Data Centers." They are treating workforce data as IT infrastructure—requiring complex databases to track licensure, vacancy, and pipeline metrics in real-time.

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# 5. Conclusion

The Rural Health Transformation Program is a "state-architected" reconstruction of the rural safety net. The flow of the $50 billion is not uniform; it is highly idiosyncratic to each state's political and administrative culture.

- In **Wyoming**, the money flows into a trust fund for hospital solvency.
- In **Indiana**, it flows to a high-tech command center and regional coalitions.
- In **South Carolina**, it flows into a Venture Capital-style "Catalyst Fund."

For Dell and other infrastructure partners, the primary customer is shifting. While individual rural hospitals remain the end-users, the **purchasing power** has moved up the chain to State Departments of Health, newly formed Regional CINs (Oklahoma), and statewide innovation hubs (Maine). Success in this market requires navigating these 50 distinct procurement environments, each with its own "Allowable Use" priorities and strategic goals.

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